

NEXTCARE MX

PRODUCT PACKET

From detection to an accountable next action

Business Bending - Week 5

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FICTIONAL DEMO | NAVIGATION SUPPORT ONLY | NOT MEDICAL ADVICE

1. Problem and point of view

My first instinct was to improve AI detection. I changed my position: the prediction is not the product. The product should be the pathway from an existing abnormal screen to a real next medical action. An abnormal result can create fear without creating an appointment, affordable treatment, or a person accountable for the handoff.

NextCare MX tests the navigation layer after detection. It is not a symptom checker, diagnostic system, medical-image reader, or treatment tool.

2. Exact user

Lucía Hernández is a fictional 55-year-old informal worker in Mexico who mainly uses WhatsApp. She receives an existing provider-approved abnormal retinal screening result. NextCare MX does not generate or reinterpret that result; it helps organize the approved next step.

3. Success definition

A fictional case can move through an accountable handoff containing Next Action, Owner, Deadline, Status, Barrier, and Escalation. After two unsuccessful automated attempts, automation stops and human review is required. Accepting ownership never falsely marks the case resolved.

4. Working slice and structured signal

Field	Demo value	Behavior
Existing result	Abnormal retinal screening	Displayed; never reinterpreted
Next action	Confirmatory ophthalmology evaluation	Provider-defined
Owner	Lucía / navigation support	Names responsibility
Deadline	18 Sep 2026	Provider-defined demo date
Barrier	Transportation	Can be selected from approved options
Attempts	0 to 2	Second failure stops automation
Escalation	Human navigator review	Required at attempt 2

5. Simulated AI role

The visibly labeled simulated AI converts approved structured information into plain-language navigation support and a barrier summary. It never diagnoses, chooses treatment, determines clinical urgency, interprets raw images, or replaces a clinician.

6. Core flow

Existing provider-approved result -> fictional-demo notice -> plain-language explanation -> structured handoff -> automated attempt 1 -> automated attempt 2 -> STOP AUTOMATION -> HUMAN REVIEW REQUIRED -> human accepts ownership -> ACCEPTED - STILL UNRESOLVED.

7. Actor responsibilities

Actor	Responsibility
Patient	Reviews next action and reports a barrier
System	Tracks handoff state, counts attempts, stops automation
Simulated AI	Explains only approved information
Navigator	Reviews escalation and may accept unresolved ownership
Clinician	Defines result, urgency, next medical action, and treatment

8. Benchmark and localization

The product benchmark is Aidoc Patient Management because it treats findings as the beginning of a follow-up workflow rather than its endpoint. NextCare MX tests a lighter, Mexico-oriented, phone-first navigation layer without assuming a deeply integrated hospital EHR environment.

9. Scope cut

Building

- One fictional case and one existing approved result
- Structured handoff state and barrier selector
- Two-attempt escalation and human acceptance
- Responsive patient and navigator interface
- Simulated, visibly labeled AI navigation text

Not building

- Symptom checking, diagnosis, image interpretation, risk scoring, urgency or treatment decisions
- Real patient storage, hospital integration, insurance approval, payment, or appointment guarantees
- Production healthcare capacity or clinical care

10. Architecture and security floor

Layer	Choice	Reason
Interface	HTML / CSS / JavaScript	Small, free, runnable prototype
State	Validated in-browser object	Structured signal changes behavior
AI	Simulated and labeled	No medical model claim or API key
Persistence	None	Minimizes privacy risk
Hosting	Static deployment target	Public demo link

Layer	Choice	Reason
Evidence	GitHub branch and tests	Traceable implementation

No secrets or real patient data are used. Inputs are restricted to predefined options and transitions. Any future live model must use server-side validation and environment-managed keys.

11. Blueprint conditions and shadow clause

- Clinical boundary: existing approved result only; clinicians retain diagnosis, urgency, and treatment.
- Owned handoff: every case carries action, owner, deadline, status, barrier, and escalation.
- Human escalation: stop automation after two unsuccessful attempts.
- Honest access: never promise appointment, coverage, affordability, or capacity.
- Measure completed or accepted handoffs, not messages sent.
- Coordination cannot replace care or become surveillance; minimize data and never assign family labor automatically.

12. Test plan and evidence

- Initial state shows all fields, fictional-demo label, clinical boundary, and simulated-AI label.
- Attempt 1 increments the counter and keeps automation available.
- Attempt 2 disables automation and requires human review.
- Human acceptance displays ACCEPTED - STILL UNRESOLVED.
- Barrier selection updates the case context.
- Responsive layout collapses to one column below 900 px.

The repository includes a passing eight-check mechanical test, a documented manual scenario, one bug and fix, a persona walkthrough, and ten dedicated Week 5 commits.

13. Long view

If the slice works, NextCare MX could become a lightweight navigation layer connecting existing screening programs to verified next clinical actions across fragmented healthcare pathways. It should connect validated screening channels, clinics, pharmacies, public routes, and human navigators without pretending software creates capacity. Success is whether a person completes the crossing from detection to care without being lost afterward.